REFERENCE CARD FOR WHO EMERGENCY UNIT FORM: GENERAL

DATES/TIMES: Do not leave dates/times blank. Where unknown, write UNK. If more than one calendar is used in your setting, all dates must be converted to Gregorian calendar and times converted to 24-hour format by data clerk before it is entered into registry.

MASS CASUALTY: Check box if patient part of a mass casualty event.

AGE: If age unknown, circle category: Infant if appears <1 year of age, Child if 1-18 years, or Adult.

SEX: Biological sex, differs from patient defined Gender category.

OCCUPATION: Be as specific as possible (eg. farm laborer or farm manager instead of farming).

PATIENT RESIDENCE: Be as specific as possible. Note if homeless, migrant worker, other.	**RACE/ ETHNICITY:** In the patient's own words.	**INTERPRETER:** Ask whether required.

AMBULATORY: Note if the patient is able to walk. If unable, note if this is a new/acute problem or if it is an old/chronic problem.

DISABILITY: Any developmental, physical or intellectual problem that impacts the patient's ability to perform activities independently.

SAFE AT HOME: Ask about violence in the home.	**CONTACT:** Ask who the patient would like to designate.
VACCINATIONS: Ask if up to date. Review card if available.	**SUBSTANCE USE:** Ask about current or prior substance use.

PREGNANT: Ask if patient knows if she is pregnant, designate whether formal testing has been done.

G: Gravida – number of pregnancies.	**P**: Para – number of live births.
CHIEF COMPLAINT: Always in the patient's own words.	**ALLERGIES:** Ask about any medication, drug, or food allergies.

DEAD ON ARRIVAL: Use ONLY if NO signs of life on arrival.

NORMAL VITAL SIGNS FOR ALL: SpO_2 >92% on RA, Temp 36˚C - 38˚C

Adult: Pulse 60-100 bpm, RR 10-20, SBP >90

Pain Score: Ask the patient to rate their pain 0 (none) to 10 (severe or worst).

Paediatric:

AGE	RESPIRATORY RATE
<2 months	40-60 breaths per minutes
2-11 months	25-50 breaths per minute
1-5 years	20-40 breaths per minute

AGE	PULSE RATE RANGE
0-1	100-160
1-3	90-150
3-6	80-140

*Record O_2 saturation and amount/route of O_2, eg. 94% on 2L by NC

HIGH RISK RED SIGNS: Check box if patient meets any criteria.	**TRIAGE CATEGORY:** Record color designation using standard tool.

TREATING PROVIDER ASSESSMENT Date and time of first assessment of patient by medical provider at current facility.

PRIMARY SURVEY

Normal Airway: (Check box if) •Patent (speaking normally) •NO signs of obstruction, stridor or angioedema		•OPA = oropharyngeal airway •NPA = nasopharyngeal airway •LMA = laryngeal mask airway •BVM=bag valve mask •ETT = endotracheal tube
Normal Breathing: • Effort normal • Sounds clear	**Abnormal Breathing:** •RR <10 or >20 (enter N/A for spontaneous RR if sedated, paralyzed, or on ventilator) • Breath sounds: note presence with check and quality on lung fields (such as distant, wheezing, rhonchi, crackle, crepitation) • Cyanosis if present	•Record flow rate and route: Nasal = nasal cannula • Facemask = simple facemask •NRB = non-rebreather mask •BVM = bag valve mask •CPAP/BiPAP = continuous or bi-level positive airway pressure •Ventilator = mechanical ventilation •Chest tube: note location, size, and depth if inserted •Salbutamol: check box if given
Normal Circulation: •Warm & dry •Pulse strong & symmetric in all extremities	**Abnormal Circulation:** •Cool, moist, pale skin •Prolonged capillary refill > 3 seconds •Asymmetric pulses (note which is abnormal) •JVD (jugular venous distention)	Access: Document location and size •IV = peripheral intravenous •Central = central venous line •IO = intraosseous line •IVF = intravenous fluids, types include: NS = normal saline, LR = Lactated Ringer's, Other (write name) •Adrenaline: check box if given
Normal Disability: •Alert (A) •Moves all extremities •Oriented to person, place, time •Pupils symmetric, reactive, and mid-range	**Abnormal Disability:** • Responds to V = Verbal or P = Painful Stimulus or is U = Unconscious • Motor or sensory deficit: note location • Pupils: indicate size and reactivity	•Blood Glucose: document level, abnormal if <65 mg/dL or <3.5 mmol/L •Interventions: note if glucose (any form), antidote (such as naloxone or atropine or other), antiepileptic, antibiotic, or other medication is administered

HISTORY OF PRESENT ILLNESS: Symptoms, time course, exacerbating and alleviating factors, prior episodes & prior interventions, including any primary health care

REVIEW OF SYSTEMS (*If patients do not have any of these symptoms, mark as normal)*	
General: Fever, chills, night sweats, fatigue, weight loss **Head/Ears/Eyes/Nose/Throat (HEENT):** Vision changes, discharge (eye/ear), pain (eye/ear), nose bleeds, mucosal lesions, difficulty swallowing, drooling, sore throat, dental problem, facial swelling **Respiratory:** Difficulty breathing or wheezing, cough, sputum production, bloody sputum **Cardiovascular (CV):** Chest pain or tightness, palpitations, orthopnea, edema **Gastrointestinal (GI):** Anorexia, abdominal pain, nausea, vomiting, vomiting blood, diarrhea, blood in stool, black/tarry stool **Genitourinary (GU):** Urination (difficulty, pain, frequency, blood), incontinence, flank pain, genital lesions **Female Reproductive:** Vaginal bleeding or discharge, abnormal menses, pelvic pain (*If pregnant* – Decreased fetal movement, contractions, leakage of fluid**)**	**Male Reproductive:** Penile discharge or pain, testicular pain, priapism **Skin:** Rash, itching, jaundice, ulcers **Musculoskeletal:** Myalgia, joint pain/swelling **Hematologic:** Lymphadenopathy, easy bruising **Neurologic (Neuro):** Headache, syncope, focal weakness, numbness, dizziness, lightheadedness, speech problems, imbalance **Psychiatric:** Hallucination, agitation, homicidal thoughts, suicidal thoughts, depression, anxiety **Pediatric Specific:** Unable to feed, decreased activity, decreased urine, vomiting everything, convulsions, excessive irritability
PAST MEDICAL HISTORY	
Past Medical History: •HTN = Hypertension •DM = Diabetes •COPD = Chronic Obstructive Pulmonary Disease •Other: list conditions not noted, such as heart disease, stroke, asthma, cancer, and HIV/AIDS) **Past Surgeries:** include type and date	**Medications:** include anticoagulants, prescription medications, traditional medicines, herbs, and supplements **Family History**: include early death, known heart disease, cancer, epilepsy, or others
Normal Exam (Check Normal only if NO abnormal findings as below are present)	
General: Well-developed, well-nourished, awake, alert **Neuro/Psychiatric:** Oriented X3, CN intact, no focal weakness or sensory deficits**,** normal mood/affect/behavior/thought content **HEENT:** Normocephalic, atraumatic, eyes **-** pupils equal and reactive, extra ocular movements intact, conjunctivae normal **Neck:** Trachea midline, neck supple, range of motion normal, no lymphadenopathy **Cardiac:** Normal rate and rhythm, strong pulses, normal sounds	**Respiratory:** Normal effort and breath sounds **Abdominal:** Soft and non-tender, bowel sounds normal **Pelvis/GU/Rectal:** External genitals normall, no costovertebral angle (CVA) tenderness **MSK:** Range of motion nml, no deformities **Skin:** Warm, intact, capillary refill ≤3 sec **Lymph:** No lymphadenopathy
Abnormal Exam Findings (Always specify right or left when needed to clarify abnormal finding)	
General: Distressed, malnourished (if suspect obtain MUAC), diaphoretic, uncooperative, sedated, lethargic **Neuro/Psychiatric:** Neuro **-** Disoriented, CN deficit, focal sensory or motor deficit, abnormal gait or coordination, tremors, seizure activity, Kernig/Brudzinski sign, abnormal rectal tone. Psych- Suicidal, depressed, homicidal, delusional, agitated, hallucinating, abnormal speech **HEENT:** Dry mucus membranes, tonsillar exudate, abnormal fontanelle, ear discharge, oral lesions, facial swelling. Eyes-conjunctiva pale, peri-orbital lesion, abnormal ocular movements, scleral jaundice, eye discharge, pupils unequal and/or slow or non-reactive **Neck:** Neck stiffness, JVD, carotid bruit, neck mass, tracheal deviation **Cardiac:** Distant heart sounds, systolic or diastolic murmur, S3 or S4 gallop, friction rub, irregular pulse	**Respiratory:** Absent breath sounds, decreased breath sounds, crackles, wheezes **Abdominal:** Distension, tenderness, rebound, guarding, ascites, hepatomegaly, splenomegaly, mass **Pelvis/GU/Rectal:** Penile discharge, testicular mass or tenderness, CVA tenderness**,** vaginal bleeding or discharge, cervical motion tenderness, adnexal tenderness, blood or dark stool on rectal exam, *If pregnant* - No fetal heart rate **MSK:** Joint swelling, decreased ROM or strength, scoliosis, kyphosis, spine tenderness **Skin:** Rash, lesion, ulcer, pustules, bruising, petechiae, poor turgor, capillary refill > 3 seconds **Lymph:** Adenopathy (head, cervical, supraclavicular, axillary or inguinal), lymphedema
DIAGNOSTICS & PROCEDURES: check if ordered, note results **Rapid Diagnostics:** include items such as rapid malaria, HIV **ECG:** Electrocardiogram **Other:** include items such as lactate, amylase, lipase, PT/INR, PTT, CK, and cultures [blood, CSF or urine]) with result **Imaging**: Specify radiograph type (XR, CT, U/S), location and results. If study needed but not available, write this in other. **Procedure:** include type, location, outcome, and time	**MEDICATIONS:** **Fluids/Medications:** list Blood product type (eg. PRBC, platelets) and number of units, write medication name/dose in appropriate category if applicable (eg. Opioid Analgesia: Morphine 4 mg) •Other: Vasopressors, post-intubation gtt, etc. •Other: Diagnostic peritoneal lavage, regional block, central line placement (if not noted in "Circulation" section), suprapubic catheterization, cricothyroidotomy, foreign body removal, etc.
ASSESSMENT AND PLAN: include summary and differential diagnosis AND plan for diagnostics, intervention, consults	
REASSEMENT: include time, vital signs, and condition (mark condition same or changed, if changed include description)	
DISPOSITION: include whether WHO medical emergency checklist was completed, date and time of ED departure, updated vital signs (VS), list of diagnoses, and check box for destination. Include the accepting provider if applicable once handover performed. Confirm that the plan of care, including follow-up was discussed with all discharged patients. Specify cause of death, but do not write cardiac or respiratory failure/arrest. Instead, use precise terms such as "pneumonia", "organophosphate poisoning", or "suicide".	
Document all providers engaged in the patient's care including through shift handovers.	